

SENATE No. 711

The Commonwealth of Massachusetts

PRESENTED BY:

Julian Cyr

To the Honorable Senate and House of Representatives of the Commonwealth of Massachusetts in General Court assembled:

The undersigned legislators and/or citizens respectfully petition for the adoption of the accompanying bill:

An Act relative to rate equity for community health centers.

PETITION OF:

NAME:	DISTRICT/ADDRESS:	
<i>Julian Cyr</i>	<i>Cape and Islands</i>	
<i>Joanne M. Comerford</i>	<i>Hampshire, Franklin and Worcester</i>	<i>1/28/2025</i>
<i>Paul R. Feeney</i>	<i>Bristol and Norfolk</i>	<i>1/31/2025</i>
<i>David T. Vieira</i>	<i>3rd Barnstable</i>	<i>2/24/2025</i>
<i>Liz Miranda</i>	<i>Second Suffolk</i>	<i>3/21/2025</i>
<i>Michael O. Moore</i>	<i>Second Worcester</i>	<i>3/28/2025</i>
<i>James K. Hawkins</i>	<i>2nd Bristol</i>	<i>4/14/2025</i>
<i>John F. Keenan</i>	<i>Norfolk and Plymouth</i>	<i>4/24/2025</i>
<i>Joan B. Lovely</i>	<i>Second Essex</i>	<i>5/27/2025</i>
<i>Vanna Howard</i>	<i>17th Middlesex</i>	<i>5/27/2025</i>
<i>Adam J. Scanlon</i>	<i>14th Bristol</i>	<i>5/27/2025</i>
<i>Manny Cruz</i>	<i>7th Essex</i>	<i>5/27/2025</i>
<i>Pavel M. Payano</i>	<i>First Essex</i>	<i>7/21/2025</i>
<i>James B. Eldridge</i>	<i>Middlesex and Worcester</i>	<i>12/1/2025</i>

SENATE No. 711

By Mr. Cyr, a petition (accompanied by bill, Senate, No. 711) of Julian Cyr, Joanne M. Comerford, Paul R. Feeney and David T. Vieira for legislation to require prospective payment system methodology for reimbursement to community health centers. Financial Services.

[SIMILAR MATTER FILED IN PREVIOUS SESSION
SEE SENATE, NO. 620 OF 2023-2024.]

The Commonwealth of Massachusetts

In the One Hundred and Ninety-Fourth General Court
(2025-2026)

An Act relative to rate equity for community health centers.

Be it enacted by the Senate and House of Representatives in General Court assembled, and by the authority of the same, as follows:

1 SECTION 1. Chapter 32A of the General Laws, as appearing in the 2022 Official
2 Edition, is hereby amended by inserting after section 33 the following new section:-

3 Section 34. (a) For the purposes of this section, the following terms shall have the
4 following meanings unless the context clearly requires otherwise:

5 “Federally Qualified Health Center”, any entity receiving a grant under 42 U.S.C. 254B.

6 “Federally Qualified Health Center Services”, as such term is defined in 42 U.S.C.
7 1396d(a)(2)(C), and as further defined in 101 CMR 304.00.

8 (b) Notwithstanding any general or special law to the contrary, the Commission shall
9 ensure that the rate of payment for any Federally Qualified Health Center services provided to a

patient by a community health center, shall be reimbursed in an amount at least equivalent to the annual aggregate revenue that the health center would have received if reimbursed by MassHealth pursuant to methodology that conforms with 42 U.S.C. § 1396a(bb) and 1396b(m)(2)(A)(ix) as they appear in Title 42 of the United States Code as of January 1, 2025.

SECTION 2. Chapter 118E of the General Laws, as appearing in the 2022 Official Edition, is hereby amended by inserting after section 13d $\frac{1}{2}$ the following new section:-

Section 13d $\frac{3}{4}$. (a) For purposes of this section, the term “community health center” shall mean any entity reimbursed as a community health center under this chapter.

(b) Notwithstanding any general or special law to the contrary, reimbursement for community health centers under this chapter, shall be through a methodology that conforms with 42 USC § 1396a(bb) and 1396b(m)(2)(A)(ix) as appearing in Title 42 of the United States Code as of January 1, 2025.

SECTION 3. Chapter 175 of the General Laws, as appearing in the 2022 Official Edition, is hereby amended by inserting after section 47UU the following new section:-

Section 47VV. (a) For the purposes of this section, the following terms shall have the following meanings unless the context clearly requires otherwise:

“Federally Qualified Health Center”, any entity receiving a grant under 42 U.S.C. 254B.

“Federally Qualified Health Center Services”, as such term is defined in 42 U.S.C. 1396d(a)(2)(C), and as further defined in 101 CMR 304.00.

(b) Notwithstanding any general or special law to the contrary, insurers organized under this chapter shall ensure that the rate of payment for any Federally Qualified Health Center

31 services provided to a patient by a community health center, shall be reimbursed in an amount at
32 least equivalent to the annual aggregate revenue that the health center would have received if
33 reimbursed by MassHealth pursuant to methodology that conforms with 42 U.S.C. § 1396a(bb)
34 and 1396b(m)(2)(A)(ix) as they appear in Title 42 of the United States Code as of January 1,
35 2025.

36 (c) The Division of Insurance shall issue regulations governing issuance of payments to
37 community health centers to conform with this section. The Division of Insurance shall consult
38 with MassHealth to receive technical assistance regarding the per visit payment rate for each
39 individual Federally Qualified Health Center for a given year. MassHealth shall provide the
40 Division of Insurance with a proxy rate for any Federally Qualified Health Center who has not
41 received an individual prospective payment system rate and the Division of Insurance shall make
42 available to health plans upon request the necessary prospective payment system rate information
43 regarding their contracted Federally Qualified Health Centers such that the health plan can
44 ensure compliance with this requirement. The Division of Insurance shall promulgate regulations
45 no later than January 1, 2027 in order to implement the provisions of this Chapter.

46 (d) Any entity licensed by the Division of Insurance and providing reimbursement to
47 federally qualified health centers for services provided to patients, including, but not limited to,
48 non-profit hospital service corporations, medical service corporations, dental service
49 corporations, health maintenance organizations, and preferred provider organizations, or any
50 other entity not specifically enumerated hereunder licensed by the Division of Insurance and
51 providing reimbursement to federally qualified health centers for services provided to patients,
52 shall submit an annual report to the Division of Insurance as a condition of their licensure
53 evidencing that the total reimbursement to Federally Qualified Health Centers for services

provided to patients in the prior year was equivalent to the annual aggregate revenue the health center would have received if reimbursed by MassHealth.

SECTION 4. Chapter 176A of the General Laws, as appearing in the 2022 Official Edition, is hereby amended by inserting after Section 38 the following new section:-

Section 39. (a) For the purposes of this section, the following terms shall have the following meanings unless the context clearly requires otherwise:

“Federally Qualified Health Center”, any entity receiving a grant under 42 U.S.C. 254B.

“Federally Qualified Health Center Services”, as such term is defined in 42 U.S.C. 1396d(a)(2)(C), and as further defined in 101 CMR 304.00.

(b) Notwithstanding any general or special law to the contrary, any corporation organized under this chapter shall ensure that the rate of payment for any Federally Qualified Health Center services provided to a patient by a community health center, shall be reimbursed in an amount at least equivalent to the annual aggregate revenue that the health center would have received if reimbursed by MassHealth pursuant to methodology that conforms with 42 U.S.C. § 1396a(bb) and 1396b(m)(2)(A)(ix) as they appear in Title 42 of the United States Code as of January 1, 2025.

SECTION 5. Section 1 of Chapter 176B of the General Laws, as appearing in the 2022 Official Edition, is hereby amended by inserting after the definition of “Dependent” the following new definitions:-

“Federally Qualified Health Center”, any entity receiving a grant under 42 U.S.C. 254B.

“Federally Qualified Health Center Services”, as such term is defined in 42 U.S.C. 1396d(a)(2)(C), and as further defined in 101 CMR 304.00.

SECTION 6. Chapter 176B of the General Laws, as so appearing is hereby further amended by inserting after Section 25 the following new section:-

Section 26: (a) Notwithstanding any general or special law to the contrary, any medical service plan organized under this chapter shall ensure that the rate of payment for any Federally Qualified Health Center services provided to a patient by a community health center, shall be reimbursed in an amount at least equivalent to the annual aggregate revenue that the health center would have received if reimbursed by MassHealth pursuant to methodology that conforms with 42 U.S.C. § 1396a(bb) and 1396b(m)(2)(A)(ix) as they appear in Title 42 of the United States Code as of January 1, 2025.

SECTION 7. Section 1 of Chapter 176E of the General Laws, as appearing in the 2022 Official Edition, is hereby amended by inserting after the definition of “Dental Service Corporation” the following new definitions:-

“Federally Qualified Health Center”, any entity receiving a grant under 42 U.S.C. 254B.

“Federally Qualified Health Center Services”, as such term is defined in 42 U.S.C. 1396d(a)(2)(C), and as further defined in 101 CMR 304.00.

SECTION 8. Said Chapter 176E is further amended by inserting after section 15A the following new section:-

Section 15B. (a) Notwithstanding any general or special law to the contrary, any Dental Service Corporation organized under this chapter shall ensure that the rate of payment for any

Federally Qualified Health Center services provided to a patient by a community health center, shall be reimbursed in an amount at least equivalent to the annual aggregate revenue that the health center would have received if reimbursed by MassHealth pursuant to methodology that conforms with 42 U.S.C. § 1396a(bb) and 1396b(m)(2)(A)(ix) as they appear in Title 42 of the United States Code as of January 1, 2025.

SECTION 9. Section 1 of Chapter 176G of the General Laws, as appearing in the 2022 Official Edition, is hereby amended by inserting after the definition of “Evidence of Coverage” the following new definitions:-

“Federally Qualified Health Center”, any entity receiving a grant under 42 U.S.C. 254B.

“Federally Qualified Health Center Services”, as such term is defined in 42 U.S.C. 1396d(a)(2)(C), and as further defined in 101 CMR 304.00.

SECTION 10. Said Chapter 176G is further amended by inserting after section 33 the following new section:-

Section 34. (a) Notwithstanding any general or special law to the contrary, any Health Maintenance Organization organized under the laws of the Commonwealth shall ensure that the rate of payment for any Federally Qualified Health Center services provided to a patient by a community health center, shall be reimbursed in an amount at least equivalent to the annual aggregate revenue that the health center would have received if reimbursed by MassHealth pursuant to methodology that conforms with 42 U.S.C. § 1396a(bb) and 1396b(m)(2)(A)(ix) as they appear in Title 42 of the United States Code as of January 1, 2025.

SECTION 11. Section 1 of Chapter 176I of the General Laws, as appearing in the 2022 Official Edition, is hereby amended by inserting after the definition of “Emergency Care” the following new definitions:-

“Federally Qualified Health Center”, any entity receiving a grant under 42 U.S.C. 254B.

“Federally Qualified Health Center Services”, as such term is defined in 42 U.S.C. 1396d(a)(2)(C), and as further defined in 101 CMR 304.00.

SECTION 12. Said Chapter 176I, as so appearing, is further amended by inserting after section 13 the following new section:-

Section 14. (a) Notwithstanding any general or special law to the contrary, any preferred provider contract shall ensure that the rate of payment for any Federally Qualified Health Center services provided to a patient by a community health center, shall be reimbursed in an amount at least equivalent to the annual aggregate revenue that the health center would have received if reimbursed by MassHealth pursuant to methodology that conforms with 42 U.S.C. § 1396a(bb) and 1396b(m)(2)(A)(ix) as they appear in Title 42 of the United States Code as of January 1, 2025.

SECTION 13. Chapter 15A of the General Laws, as appearing in the 2022 Official Edition, is hereby amended by inserting after section 18 the following new section:-

Section 18A. (a) For the purposes of this section, the following terms shall have the following meanings unless the context clearly requires otherwise:

“Federally Qualified Health Center”, any entity receiving a grant under 42 U.S.C. 254B.

135 “Federally Qualified Health Center Services”, as such term is defined in 42 U.S.C.
136 1396(a)(2)(C), and as further defined in 101 CMR 304.00.

137 (b) Notwithstanding any general or special law to the contrary, any student health
138 insurance program or plan authorized under Section 18 of Chapter 15A shall ensure that the rate
139 of payment for any Federally Qualified Health Center services provided to a patient by a
140 community health center, shall be reimbursed in an amount at least equivalent to the annual
141 aggregate revenue that the health center would have received if reimbursed by MassHealth
142 pursuant to methodology that conforms with 42 U.S.C. § 1396a(bb) and 1396b(m)(2)(A)(ix) as
143 they appear in Title 42 of the United States Code as of January 1, 2025.